Society for
Digital Mental Health

June 16, 2025

Via Docket Submission

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

***Subject*: Response for request for information (RFI) seeking input from the public regarding the market of digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure**
Docket Number: CMS–0042–NC

Dear Administrator Oz,

The Society for Digital Mental Health (SDMH) is a non-profit organization with membership from academic research institutions, healthcare systems, payers, digital mental health companies, and patient advocacy. The mission of the SDMH is to: 1) support knowledge generation and sharing around the design, effectiveness, use, implementation, and dissemination of digital mental health products and services; 2) foster the development and application of evidence standards and evidence-based best practices for sustainable, effective implementation of digital mental health technologies across different care delivery settings and models; and 3) promote the equitable use of digital mental health products and services, with a focus on benefiting underserved and at-risk communities and reducing health disparities.

SDMH is pleased that CMS responded favorably to our prior comments (**Response to Request for Information on Digital Therapies, Such as, But Not Limited to, Digital Cognitive Behavioral Therapy at 88 FR 52262, 52369-52371,** dated 9/7/23**)** and (**Response to Medicare and Medicaid Programs; CY 2025 Payment Policies under the Physician Fee Schedule and Other Changes to Part B Payment and Coverage Policies specifically reimbursement for DMHT**, dated 8/29/24**)** and are appreciative of the diligence and commitment by CMS to foster a collaborative approach to closing gaps in care for mental health disorders.

SDMH is committed to advancing the adoption, efficacy, and ethical deployment of digital mental health solutions across healthcare systems, particularly in supporting vulnerable populations such as Medicare beneficiaries and their caregivers. We welcome the opportunity to provide feedback on

CMS's consideration of how to encourage greater uptake and responsible oversight of digital health tools. We are grateful for the opportunity to respond as an organization that represents a diverse group of stakeholders that can attest to the transformative impact that digital mental health solutions can have on patient care and outcomes.

Society for Digital Mental Health Comments

PC 5: What can CMS and its partners do to encourage patient and caregiver interest?

To foster trust and drive engagement among Medicare beneficiaries and caregivers, CMS should focus on three core pillars: *education, trust-building,* and *integration into care workflows*. We recommend:

- *National campaigns for digital health literacy:* Partner with community-based organizations, libraries, senior centers, and health plans to roll out education campaigns focused on teaching digital health basics, reducing stigma, and building trust in technology.

- *Patient-centered design evaluations:* Support development of digital health tools that meet adults' usability needs. CMS could incentivize developers to meet co-design standards that include direct input from Medicare users.

- *Clinical integration incentives:* Encourage providers to incorporate digital health tools into care pathways and patient conversations. While informing patients of the availability of digital mental health treatments (DMHTs) can theoretically reach large numbers of patients, this practice alone results in low DMHT uptake. Provider endorsement has been shown to be critical for promoting patient adoption.[1]

- *Support healthcare systems in cost-effective methods for supporting patients in registering and engaging in DMHTs*. Research and experience in DMHT implementation has shown that while many patients are interested in DMHTs, they rarely follow-up on referrals without further assistance. There are several reasons for low patient uptake on DMHT referrals, including forgetfulness, low motivation and or anxiety related to the mental health condition, unfamiliarity with DMHT services, and low digital literacy. Particularly in medical settings such as primary care, where most common mental health treatment is provided, clinicians typically do not have the time to assist patient onboarding or support longer term engagement.

We applaud CMS for recognizing the need for patient support services in the current HCPCS Codes (G-0553, G-0554). However, these codes require a licensed qualified provider, which presents several obstacles:

[1] Graham AK, Greene CJ, Powell T, et al. Lessons learned from service design of a trial of a digital mental health service: Informing implementation in primary care clinics. Translational behavioral medicine. Aug 7 2020;10(3):598-605. doi:10.1093/tbm/ibz140

1. Behavioral health providers are already in short supply in many care settings. Requiring support to be provided by a qualified provider reduces the amount of time that behavioral health providers have to provide traditional care.
2. Many qualified behavioral health providers are also unfamiliar with DMHT services and are not prepared to address issues related to digital literacy.

Allowing non-licensed digital navigators or peer-support specialists to provide support for onboarding and continued engagement would better meet the needs of both patients and healthcare systems. There is overwhelming evidence both in the randomized controlled trial literature as well as in implementation literature that effective, safe support for onboarding and monitoring can be provided by trained, non-licensed providers such as coaches or peer-supporters.[2] CMS has recognized the value, utility, and safety of trained, certified auxiliary personnel, such as peer specialists, in providing remote care and telebehavioral health services through codes such as G0140, G0146, and H0038. We strongly urge CMS to consider extending these codes to allow support for DMHT services. This would support integration of DMHT services in high volume care systems where clinician time is scarce, such as primary care, family medicine, and community-based healthcare systems.

Using trained, certified peer-specialists also supports adaptation to the needs of different communities. It is unlikely that the marketplace can support large numbers of carefully tailored tools that can support the specific needs of different communities (e.g., rural vs. urban, the wide number of possible comorbid medical problems, religious beliefs, socio-economic differences). DMHT services can utilize peers from the patient communities to support tailoring of DMHT service use to better meet the needs of individuals and their communities.

PC-5 a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process?

While CMS does not function as a regulator of digital health product authorization, we believe CMS can play a crucial role by working with developers and researchers to guide best evidence standards to guide clinical and business decision-making. This would support providers and patients in identifying trusted tools.

---

[2] Linardon J, Cuijpers P, Carlbring P, Messer M, Fuller-Tyszkiewicz M. The efficacy of app-supported smartphone interventions for mental health problems: a meta-analysis of randomized controlled trials. World Psychiatry. Oct 2019;18(3):325-336. doi:10.1002/wps.20673

Graham AK, Greene CJ, Kwasny MJ, et al. Coached Mobile App Platform for the Treatment of Depression and Anxiety Among Primary Care Patients: A Randomized Clinical Trial. JAMA Psychiatry. Sep 1 2020;77(9):906-914. doi:10.1001/jamapsychiatry.2020.1011

Jaso-Yim B, Eyllon M, Sah P, et al. Evaluation of the impact of a digital care navigator on increasing patient registration with digital mental health interventions in routine care. Internet Interv. Dec 2024;38:100777. doi:10.1016/j.invent.2024.100777

SDMH, working with seven large healthcare systems, identified the following criteria that are important in evaluating digital mental health products[3]:

- *Clinical efficacy data* from robust, well-designed randomized controlled trials (RCTs) are the gold standard evidence to demonstrate that a digital mental health tool is effective at improving mental health symptoms.

- *Real-world data (RWD)* is an important complement to RCT data. While RCT data provides the strongest proof-of-principle support for the efficacy and effectiveness, RCTs often suffer from sample biases, as they require considerable time and effort for the completion of research procedures (e.g., consent and often substantial assessment), thereby selecting highly motivated participants. RWD, collected in the process of care delivery, does not involve any additional patient research-related effort. Thus, RWD offers evidence that the digital mental health service can provide value under real world circumstances.

- *Clinical review* by the healthcare system or clinics providers is commonly conducted to ensure the digital mental health product employs valid clinical strategies and aligns with providers' values and clinical approaches.

- *Usability for the intended population*. Many healthcare systems engage their patients in evaluating the usability of the product.

- *Health equity review* is conducted to ensure that the digital mental health service is accessible to all patients, and does not exclude specific groups (e.g., rural, low income, patients with disabilities or with low connectivity).

- *Data privacy and security compliance* (HIPAA, cybersecurity standards) must be reviewed.

- *Interoperability* with EHR systems and potentially with CMS infrastructure to ensure data integration.

What technology or policy recommendations can increase adoption?

- *Reimbursement adjustments:* While the new HCPCS codes for digital mental health treatment have been developed, it does present some challenges to widespread adoption and are not representative of all high-quality products within this space. To ensure uptake of these services, SDMH advocates for expanded reimbursement for other categories of DMHT that demonstrate safety and effectiveness outside of CFR 882.5801.

- *Expanding reimbursement to include a broader range of evidence-based digital mental health products:* To support patient access and provider flexibility, we recommend expanding reimbursement to include evidence-based digital mental health products beyond those with FDA clearance. Limiting reimbursement only to FDA-cleared products excludes many rigorously validated interventions—such as those supported by randomized controlled trials

[3] Mohr DC, Silverman AL, Youn SJ, et al. Digital mental health treatment implementation playbook: successful practices from implementation experiences in American healthcare organizations. Front Digit Health. 2025;7:1509387. doi:10.3389/fdgth.2025.1509387

(RCTs)—that address a wide range of mental health conditions. Currently, FDA-cleared digital therapeutics focus on a narrow set of conditions (e.g., depression, panic disorder, insomnia), leaving significant gaps in care for other prevalent or complex mental health needs.Broader reimbursement would enable providers to build formularies that better reflect the range of patient populations, including those with unique cultural, linguistic, or medical needs. This approach also encourages innovation by supporting high-quality products that may not pursue FDA clearance due to cost or regulatory barriers.

- Moreover, health systems that have successfully integrated digital mental health into their care pathways typically evaluate products based on real-world evidence, clinical outcomes, and alignment with organizational goals—not FDA clearance alone. Expanding reimbursement criteria would align policy with this practice, fostering equitable access, clinical flexibility, and continuous innovation in digital mental health.

- *API expansion*: Fund the development of secure, standards-based APIs that allow seamless integration of digital health data into patient portals.

- *Beneficiary-focused product finder*: Launch a CMS-maintained directory of vetted digital health tools with plain-language summaries and user reviews.

PC-5 b. What changes would enable timely access to high quality CMS and provider generated data on patients?

To unlock the full potential of digital mental health tools, developers and providers must be able to access CMS and clinical data in a secure, timely, and structured way. We suggest:

- *Fast-track data access for validated use cases*: Create pathways for developers with approved IRB protocols or partnerships with CMS providers to access anonymized patient data sets more rapidly.

- *Real-time claims data pilots*: Fund pilot programs that allow real-time or near-real-time access to claims data for performance feedback and early signal detection.

- *Promote FHIR-based interoperability*: Ensure providers participating in Medicare are incentivized to support FHIR-based data exchange with third-party digital tools, reducing friction for integration.

PC-6. Accessibility and Usability for Medicare Beneficiaries and Caregivers

Key features to ensure adoption and usability include:

- *Simple, intuitive interfaces*: Clear navigation, large or adjustable fonts, audio instructions, and minimal cognitive load are critical for DMHT adoption and utilization.

- *Low-bandwidth and offline functionality*: Many older adults live in areas with unreliable internet. Offline functionality with data-syncing should be prioritized.

- *Multilingual support:* Accessibility for non-English speakers is essential for equity.

- *Live support and training*: Offer Medicare-funded access to digital health "navigators" — trained staff who can help patients set up and troubleshoot tools. As noted in our response to PC5 above, these can be peer specialists, similar to existing codes such as G0140, G0146, and H0038.

- *Integration with caregiver accounts*: Allow caregivers proxy access and ability to manage health tools on behalf of the patient.

PC-7. Real-World Data Collection on Outcomes and Costs

CMS is uniquely positioned to lead in creating a robust framework for real-world evidence (RWE) generation post-market. We recommend:

- *Standardized outcome reporting framework*: Create a core set of outcome metrics for digital mental health tools. Existing standards for patient-reported outcomes exist for many mental health conditions (e.g., PHQ-9 for depression, GAD-7 for anxiety). There are no widely accepted engagement standards for DMHTs, however some widely used metrics in the research literature may be considered, such as number of days of use for intensity and time from first use to last use for length of engagement.[4] Care coordination (or peer-support) markers, such as the number and length of support sessions would be important for basic evaluation of DMHT services.

- *Claims data linkage with app-generated data*: Enable voluntary submission of digital product usage data linked to Medicare claims data, preserving privacy through secure tokenization.

- *Provider-reported feedback loop*: Develop CMS-supported provider surveys or EHR prompts that capture clinician impressions of patient outcomes when digital tools are used.

- *Public dashboard and feedback loop:* Transparently publish aggregated RWE on digital tool effectiveness and costs, so developers, clinicians, and patients can learn from real-world patterns.

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

a. What are the current obstacles?

[4] Pratap A, Neto EC, Snyder P, et al. Indicators of retention in remote digital health studies: a cross-study evaluation of 100,000 participants. NPJ Digit Med. 2020;3:21. doi:10.1038/s41746-020-0224-8
Lipschitz JM, Pike CK, Hogan TP, Murphy SA, Burdick KE. The Engagement Problem: a Review of Engagement with Digital Mental Health Interventions and Recommendations for a Path Forward. Curr Treat Options Psych. 2023;10:119-135.

- As in our response to PC-5, we believe the requirement that support services (G0553 & G0554) be qualified healthcare providers is a substantial and unnecessary barrier to implementation, particularly in medical services where most mental health is treated, such as primary care, family medicine, and community healthcare agencies. When behavioral health services are available in those settings, they are frequently over subscribed. The extension of tele-behavioral health codes for certified peer-specialists (G0140, G0146, and H0038) allow safe and effective integration of DMHT services without affecting the availability of behavioral health services within those settings.

- The Medicare Administrative Contractors (MAC) pricing creates uncertainties and financial unpredictability for providers, particularly for those healthcare systems working in multiple states or regions. Providers also may not know rates in advance, how rates were determined or when they might change, making longer term planning difficult. The use of MACs also is associated with miscommunications and higher demands for billing information. Shifting to national pricing would reduce burden uncertainty for providers, thereby supporting adoption and integration of DMHTs.

- Currently G-0552 for the DMHT device requires billing by the provider. This may place undue burden on providers, particularly individual providers or smaller clinics. The new DMHT (Digital Mental Health Treatment) HCPCS codes specifically G0552, G0553, and G0554, are primarily designed for clinicians and mental health professionals, not pharmacies. Pharmacists play a supporting role, such as dispensing and counseling on prescription medications that are often prescribed in conjunction with DMHTs. We encourage alternative billing methods, such as using the Pharmacy Benefit Model, which would allow devices to be dispensed and adjudicated through pharmacies, similar to prescription drugs. .

b. What information should providers share with patients when using digital products in the provision of their care?

Providers should provide the following information when discussing DMHTs as a treatment option:

- Basic information on the effectiveness of the DMHT, including the possible benefits of the DMHT.

- What is expected of the patient (how frequently they should use it, and how much time it takes).

- How the DMHT fits into the care pathway. This includes what happens if the DMHT is not effective, and what other types of treatments might be offered at that point.

- What the provider's role in the treatment will be (if the provider will be receiving and reviewing symptom severity measures, or what the provider's role will be if the DMHT is not effective).

In addition, the provider or another member of the treatment team (e.g., digital navigator, coach, peer specialist) should assess factors that may interfere with the patient's ability to use the DMHT, including technological literacy, any disabilities (e.g. vision or hearing), capacity of the patient's

device or data plan to handle the requirements of the device, and any factors in the patient's environment such as connectivity problems. These factors may be barriers or may be indications of patient support services required to enable the patient to successfully engage with the DMHT service.

c. What responsibilities do providers have when recommending use of a digital product by a patient?

Providers should continue to follow patients referred to a DMHT service. As with any mental health treatment, some patients may not respond adequately. Providers should adhere to the principles of measurement-based care, tracking patient progress, and referring those patients who do not show adequate response to a higher level of care.

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Removing the barriers to effective digital health deployment in physician workflows requires coordinated efforts across policy, technology, and practice environments. Some possible solutions include:

- Expand support for interoperable, modular solutions. This could include incentivizing the use of open standards (FHIR, SMART on FHIR).
- Simplify and harmonize reporting requirements.
    - Collaborate with providers and clinical societies to streamline and align documentation and reporting requirements across programs (e.g., MIPS, HEDIS, behavioral health quality metrics).
    - Shift quality measurement toward outcomes and away from process-heavy input metrics.
- Fund innovation through grants and pilots
    - Support early-stage development and testing of workflow-enhancing tools via demonstration projects and Innovation Center pilots, especially those tailored to behavioral health.
- Foster human-centered design in tool development
    - Consider utilizing end-user testing with clinicians as a condition of participation in CMS-supported digital tool initiatives.
- Improve access to clean, structured clinical data

- Fund initiatives to support structured data capture at the point of care, including standardized templates and AI-assisted clinical note generation.
- Provide technical guidance to EHR vendors and developers on best practices for using clinical terminologies (e.g., SNOMED CT, LOINC) to improve data quality for billing and reporting.

Conclusion

The Society for Digital Mental Health supports CMS in its exploration of scalable, responsible frameworks to expand the optimized use of digital health tools. By focusing on patient-centered design, transparent evaluation, and real-world impact measurement, CMS can help build a trustworthy digital ecosystem that supports both innovation and equity in care.

We welcome further dialogue and are ready to support collaborative efforts with CMS and its partners.

Thank you for your attention to this matter.

Respectfully, on behalf of the Board of Directors for the Society for Digital Mental Health,

David Mohr, PhD

President

Society for Digital Mental Health